جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : شاديه عبد الفتاح عبد الغنى موسى
المحافظة : الجيزه **الحي / المدينة :** الهرم **العنوان :** منشاة القناطر
رقم البطاقة : 27307092101428 **رقم التليفون :** 01288059563 **تاريخ تسجيل الإستمارة :** 13-10-2026
العمر(تاريخ الميلاد) : 09-07-1973 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: شاديه عبد الفتاح عبد الغنى موسى
National ID: 27307092101428

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

Name : Shadia Abdel Fattah Abdelghani Musa	**Date : 25-07-2026 07:01**
Branch : Dokki	**Medical No. : 423975**
Ref.Physician Prof : Goustav Roussey Egypt	**Ins.Company : تعاقد مستشفيات**

ISOTOPIC BONE SCAN REPORT

Indication: Evaluation for skeletal metastases in a 53-year-old female patient with right breast carcinoma (post-surgical resection, chemotherapy, and radiotherapy). Presents with widespread bone aches.

TECHNIQUE

Whole-body planar anterior and posterior bone scintigraphy performed 2-3hours after IV injection of ^{99m}Tc-MDP.

FINDINGS

- Multiple scattered foci of abnormally increased tracer uptake throughout the cervico-dorso-lumbar spine, bilateral ribs, sternum, and pelvic bones.
- Prominent areas of hyperfixation in both femora, particularly involving the trochanteric regions.

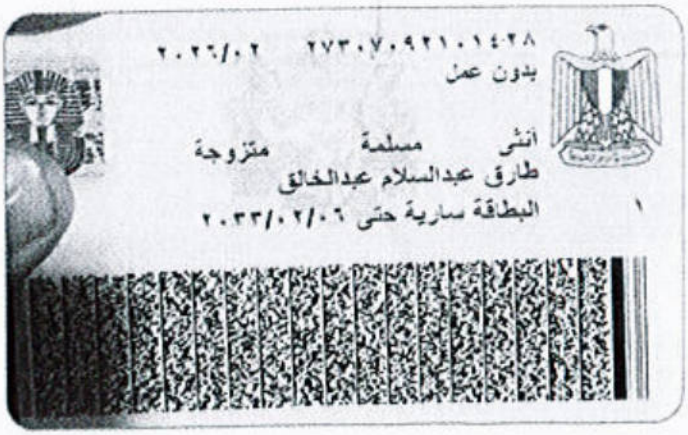
IMPRESSION

- Picture of widespread skeletal uptake pattern consistent with extensive metastatic dissemination, secondary to known primary breast malignancy, for clinical and radiological correlation.

Dr. Alaa Nagi,
MD Nuclear Medicine

Bone Scan
[[DOCTYPE:BONE_SCAN]]





National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Name: شاديه ض عبدالفتاح عبدالغنى موسى	
MR. : 9818	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		رقم طبي:	

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	

وبعد الكشف والفحوصات والمتابعة وجد انه

LABC T4DN2MX FOR STARTING TTT TILL WORKUP For urgent staging + portacath insertion + echo Meanwhile start AC x 4 cycle for c1 AC after echo + labs

for cycle 1 AC >>>Echo : EF 72% ... labs >>>accepted

CAME TODAY COMPLAINING OF CONSTIPATION AND BACK PAIN FOR MRI LUMBOSACRAL WITH CONTRAST

postm rt breast cancer locally advancer ct4d n2 m1 idc 2 er 8/8 pr8/8 her22+ sish not amplified met to bone ,lung . pt started ac till met workup . on exam bilat mild ll weakness rt breast induration redness skin nodules rt axillary lymph nodes m 6x7 cm rt supraclav ln 2x2 . cts on 6/6/2026 multiple pulmonary nodules largest 19x10 ,multiple lytic and sclerotic osseous lesions bilat iliac dorsal lumbar multiple ribs for remdt pall rth bone scan meanwhile continue ac for c2 ac adia 85 endoxan 850 mri lumbar v multiple lumbar osseous lesion indentiing thecal sac

pt rec pall rth to rt breast d8-d12 ,bilat si j l2-3 mdt pending (after pt is metastaic) for c3 ac adria 85 endoxan 850 ,zometa on exam rt breast induration ulcer healed multiple skin nodules rt ax ln 4x5 rt cervical ln 2x2

pt has severe fatigue poor ps mdt for e2 fsh for ai +_ zoladex zometya /3m new cts after 4 m +_ pall rth bone scan on 25/7/2026 widespread met osseous lesions inv cervical dl vert ribs sternum pelvic bone on exam rt breast indurated ulcerated waiting for approval of ht e2 fsh premenop

MDT 19/7/2026 FOR FSH + LH + E2 then for AI (FEMARA) +/- ZOLADEX + Palliative RTH to the breast if bleeding or ulcerated + bone if sx zometa q3m after dental assessment CT evaluation every 4 m المريضة تلقت العلاج الاشعاعى وتم رفع قرارات العلاج تم عمل تقييم وتلقت زوميتا

6/2026 mdt: 1/AI (FEMARA) +/- ZOLADEX 2/ Palliative RTH to the breast if bleeding or ulcerated + bone if sx 3/zometa q3m after dental assessment 4/CT evaluation every 4 m #pt refusing dental ttt and have many dental problem #pt 53 y old and menopausal many years ,,,why zoladex!!!! femara +supportive *3m next visit :13/11/2026

تحريرا في ,

التاريخ :	19-09-2026	الوقت :	

الطبيب المعالج : dina makram

يعتمد ,

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

06/22/2026

MDT FORM

Name:	عبدالله عبدالفتاح Abd Elghany موسى	MRN:	9818
Age:	53 Y	ID:	
Menopausal Status:	postm	Phone:	01091228623
NRS SCORE:	3	Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

R L

BX:

ER PR HER KI67



TNM

Sonomammography:

Mets Workup:

History:

postm rt breast cancer locally advancer ct4d n2 m1 idc 2 er 8/8 pr8/8 her22+ sish not amplified met to bone ,lung . pt started ec bil met workup . on exam bilat mild ll weakness rt breast induration redness skin nodules rt axillary lymph nodes in 6x7 cm rt supraclav ln 2x2 . cts on 6/6/2026 multiple pulmonary nodules largest 19x10 ,multiple lytic and sclerotic osseous lesions bilat iliac dorsal lumbar multiple ribs

Recommendations:

MDT Outcome & Plan

FOR FSH + LH + E2 then for AI (FEMARA) +/- ZOLADEX + Palliative RTH to the breast if bleeding or ulcerated + bone if sx
zometa q3m after dental assessment

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

04/21/2026

MDT FORM

Name:	شاديه عبدالفتاح Abd Elghany مرسى	**MRN:**	9818
Age:	53 Y	**ID:**	27307092101428
Menopausal Sta	PERI	**Phone:**	01144851009
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	-
CO Morbidity:		**Reason Of MDT:**	new case

Clinical Data:

BX:

ER: 8/8 **PR:** 8/8 **HER2:** - **KI67:** 15%

Sonomammography:
LABC T4DN2MX FOR STARTING TTT TILL WORKUP

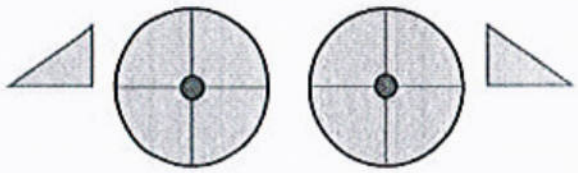
Mets Workup:

History:

Recommendations:

MDT Outcome & Plan
For urgent staging + portacath insertion
Meanwhile start AC x 4 cycles

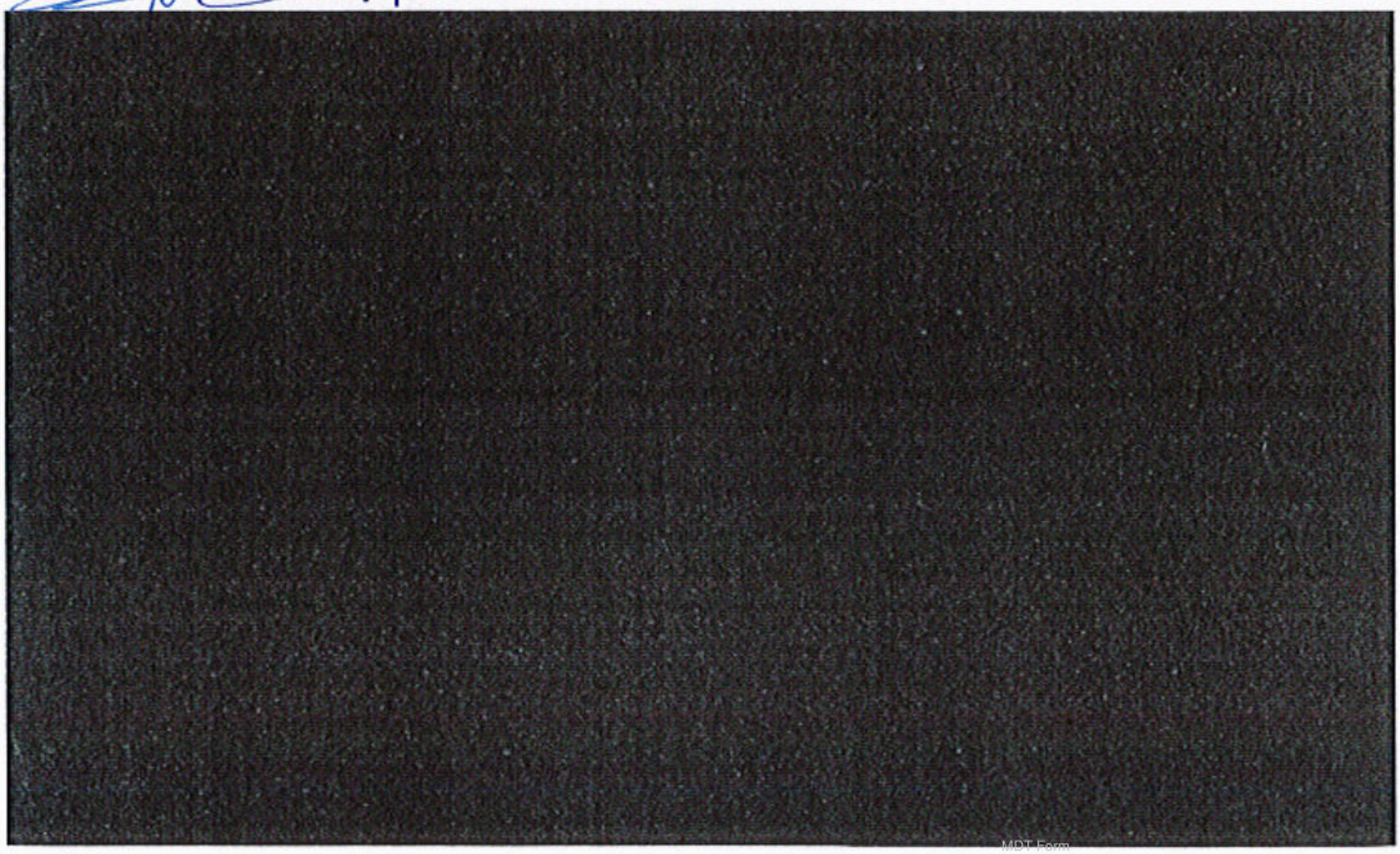
R L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ خالد جهد العزيز	د/ محمد التيراوي				



GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	:الأستاذه / شاديه عبد الفتاح عبد الغنى	Request No.	: 13005436
Patient No.	:27307092101428	Request Date	: 12/03/2026
Path No.	:701/26	Verification Date	:23/03/2026
Sex	: Female	Financial Category	: جوستاف روسي - مباشره
Age	:52 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Right locally advanced breast mass, "BIRADs 5".
Nature of Specimen: Tru-cut biopsy, collected on 12/3/2026, at 9:40 am, Cold ischemia time: 20 seconds.

Gross Examination:
- Fragmented grey white tissue cores, ranging in size between 0.2cm & 0.5 cm long, totally submitted.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
* Ductal carcinoma in situ: Absent.
* Perineural invasion: Absent.
* Angioinvasion: Indefinite.
* Necrosis: Focal.

Diagnosis:
Right breast mass, Tru-cut biopsy:
- Invasive duct carcinoma, Grade 2.
- For further IHC (ER, PR, Her2, Ki67).

Signature
Dr. Walaa Ibrahem Ghonaim.

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	:الأستاذه / شاديه عبد الفتاح عبد الغنى	Request No.	: 13005436
Patient No.	:27307092101428	Request Date	: 12/03/2026
Path No.	:701/26	Verification Date	:23/03/2026
Sex	:Female	Financial Category	:جوستاف روسي - مبادره
Age	:52 Y	Department	:Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	:.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right locally advanced breast mass, "BIRADs 5" diagnosed Invasive duct carcinoma, Grade 2.
Nature of Specimen: Tru-cut biopsy, collected on 12/3/2026, at 9:40 am, Cold ischemia time: 20 seconds.

Technique:
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for; ER clone (1E2), PR clone (SP1), Her2 clone (4B5), Ki67 clone (30-9) & SISH.

Results:
* IPT (All controls showed appropriate reactivity):
- ER: Strong nuclear reaction in 98% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 98% of tumor cells (score 8/8).
- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: Not amplified.
(Average Her2 copy No.: (3.2), Her2/CEP17 ratio: (1.3)).
- Ki67: 15%

Diagnosis:
Right breast mass , Tru-cut biopsy:
IPT:
- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Equivocal (score 2+).
- SISH: Not amplified, Group V.
(Average Her2 copy No.: (3.2), Her2/CEP17 ratio: (1.3)).
-Ki67: 15%

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

GUSTAVE ROUSSY
INTERNATIONAL GROUP

Patient Name	الأستاذه / شاديه عبد الفتاح عبد الغنى	Request No.	13005436
Patient No.	27307092101428	Request Date	12/03/2026
Path No.	701/26	Verification Date	23/03/2026
Sex	Female	Financial Category	جوستاف روسي - مبادره
Age	52 Y	Department	Out Patient
Referreing Doc.			
		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low): as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.
ISH groups:
The ratio is calculated and the results is interpreted as:
• Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
• Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).
• Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
• Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).
• Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	الأستاذه / شاديه عبد الفتاح عبد الغنى	Request No.	13005436
Patient No.	27307092101428	Request Date	12/03/2026
Path No.	701/26	Verification Date	23/03/2026
Sex	Female	Financial Category	جوستاف روسي - مبادره
Age	52 Y	Department	Out Patient
Referreing Doc.			
		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Signature
Dr. Walaa Ebrahem Ghoneim.

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED
غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient ID: 23678YT	Patient Name English: SHADYA ABEDELFATAH
Age	2026/03/10 12:56:24 PM :Report Date

Women's Health Outreach Program

Bilateral Full field Digital Mammography and high-resolution ultrasound.

Clinical summary: patient with right breast lump.

Detailed Findings

Right breast:

- Scattered fibro glandular densities (ACR density B).
- large ill-defined spiculated dense mass lesion with obscured margins and overlying suspicious microcalcifications .
- **Ultrasound** The whole right breast is a seat of locally advanced neoplastic process with overlying skin involvement showing malignant ulcer
- Multiple enlarged axillary lymph nodes at level I, II with effaced hila, largest one measures about 22 x 12.8 mm at level I.
- Few enlarged supraclavicular lymph nodes, largest one measures about 6.1 x 4.8 mm.

Left breast:

- Scattered fibro glandular densities (ACR density B).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- **Ultrasound** demonstrated prominent glandular elements of the breast tissue with mottled appearance.
- No solid or cystic lesions
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

- **Right breast (BIRADS 5): Right locally advanced mass lesion with overlying malignant ulcer with ipsilateral pathological axillary and supraclavicular lymph nodes as described. For tru cut biopsy and dynamic MRI breast study.**
- **Left breast (BIRADS 1): normal study .**

Much obliged,
Dr. marwa a . el seoud

Mammogram
[[DOCTYPE:MAMMOGRAM]]